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BIOSIMILARS RISING

The Catalyst for a New Age in LOGIC THERAPIES

Insights Corner



Month: March 2026

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- US Tariff impact on Pharma industries (**Jun, 25**)
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Additional: 1 Beautiful Bill

PRIOR ISSUES

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Unlocking Biosimilars

- What?
- Why?
- How?
- When?

of Biologic Alternatives



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Introduction

Biosimilars are biological medicines designed to closely match an already approved reference biologic in structure, function, and clinical performance. World Health Organization describes them as similar biotherapeutic products that require extensive comparison with the original medicine, and regulators like the US FDA require evidence showing no clinically meaningful differences in safety, purity, or potency

DIFFERENCE BETWEEN

	BIOLOGICS	BIOSIMILAR	BIOBETTER
FEATURE	Original, innovator biological products that establish a new therapy (e.g., first monoclonal antibody for a target)	Highly similar versions of an approved biologic with no clinically meaningful differences in safety, purity, or efficacy	Modified next-generation biologics designed to improve existing products (e.g., longer half-life, better efficacy, lower immunogenicity)
REGULATORY	Assessed via full, standalone development program with comprehensive quality, non-clinical, & clinical data under standard biologics regulations	Abbreviated pathway demonstrating high similarity to the reference product through analytical, PK/PD, and limited clinical studies	Treated as new biologics, not eligible for the biosimilar pathway, and require full development to demonstrate improvements
PATENTABILITY	Being first-in-class or first-in-target allows them to have the most robust and lengthy patent protection	Biosimilar developers can patent specific processes or formulations, but these do not prevent others from developing different biosimilars	Can build a new patent estate around the improved molecule, indications, and formulations, potentially securing stronger IP protection
COST	Highest development cost and typically highest list prices at launch due to R&D recovery and limited competition	Developed at lower cost via an abbreviated pathway and typically priced below the reference product to support adoption and payer savings	Development costs are closer to innovator biologics & they are often premium-priced if they demonstrate clear clinical or practical advantages
MARKET	Aim to introduce a new mechanism or standard of care and secure market exclusivity for as long as possible	Aim to compete with the reference biologic on price while maintaining comparable clinical performance, improving access and reducing healthcare costs	Aim to differentiate therapy with improved outcomes or convenience, targeting both switch patients and new starts rather than competing as a close copy

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Importance

Biosimilars are important because they lower the cost of biologic therapies and make advanced treatments available to more patients and health systems

1

LOWER PRICES AND SYSTEM SAVINGS

Biosimilars introduce competition for expensive brand-name biologics, which pushes down prices for both the biosimilars and the reference products. Studies in oncology, rheumatology, and other specialties show substantial reductions in total cost of care and drug spending when biosimilars are adopted, with multi-billion-dollar savings projected over time for health systems and payers.

IMPROVED PATIENT ACCESS AND ADHERENCE

Because biosimilars are typically priced lower than originator biologics, more patients can start or continue effective biologic treatment, especially in settings where cost was a major barrier. Increased affordability reduces treatment delays and medication abandonment, which supports better adherence and can prevent disease progression, particularly in low- and middle-income countries and underserved populations.

2

3

BROADER HEALTH SYSTEM BENEFITS

Savings from biosimilar use can be reinvested into other health priorities, such as funding more patients on biologics, expanding services, or adopting new innovations. As more biosimilars enter markets and policies encourage their use, competition is expected to keep biologic prices under pressure, contributing to long-term sustainability and more equitable access to high-cost therapies.

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Regulatory Pathway for Biosimilars

US & EU

	US REGULATORY PATHWAY	EU REGULATORY PATHWAY
REGULATORY AUTHORITY	Food and Drug Administration	European Medicine Agency
APPROVAL PATHWAY	Section 351(k) pathway of the public health service act	EU pharmaceutical law (e.g., Directive 2001/83/EC) and EMA centralized procedure.
INTERCHANGEABILITY	"Interchangeable" designation allowing pharmacy substitution with additional switching evidence	EMA considers biosimilars scientifically interchangeable, but there is no formal EU "interchangeable" label & each member state sets its own switching and substitution rules.
CLINICAL TRIAL	Comparative studies generally use a US-licensed reference product	Comparative studies generally use an EU-authorized reference product

FDA

EUROPEAN MEDICINES AGENCY
SCIENCE MEDICINES HEALTH

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Development Timeline: Biologics Vs. Biosimilars

BIOLOGIC DEVELOPMENT TIMELINE

DISCOVERY → DEVELOPMENT → PRECLINICAL → PHASE I → PHASE II → PHASE III

10-12 Years to develop and cost ~≥$1B

TRADITIONAL BIOSIMILAR DEVELOPMENT TIMELINE

DEVELOPMENT → PRECLINICAL → PHASE I → PHASE III

7-8 Years to develop and cost ~≥$100-300M

RECENT STREAMLINED BIOSIMILAR DEVELOPMENT TIMELINE*

DEVELOPMENT → PRECLINICAL → PHASE I

5-6 Years to develop and ~50% cost reduction

*As per the US FDA's draft guidance (proposed on Oct 29, 2025) aimed at eliminating unnecessary clinical testing

Established Scientific Foundation: With the reference biologic's mechanism of action, safety, and efficacy already validated, biosimilar developers primarily concentrate on proving high similarity instead of repeating the entire discovery process

Abbreviated Regulatory & Clinical Development: Modern biosimilar regulations rely more on advanced analytical comparability and PK/PD studies, allowing smaller clinical programs and faster approvals

Competitive Pricing & Improved Patient Access: Reduced development expenses allow biosimilars to enter the market at lower prices than originator biologics, supporting healthcare cost containment and broader patient access

Moderate Price Reduction vs. Generics: Despite being cheaper than originator biologics, biosimilar price reductions are smaller than those seen with generics due to complex manufacturing and multi-year development timelines

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Strategic Market Analysis



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Biosimilar Market: $197B Revolution

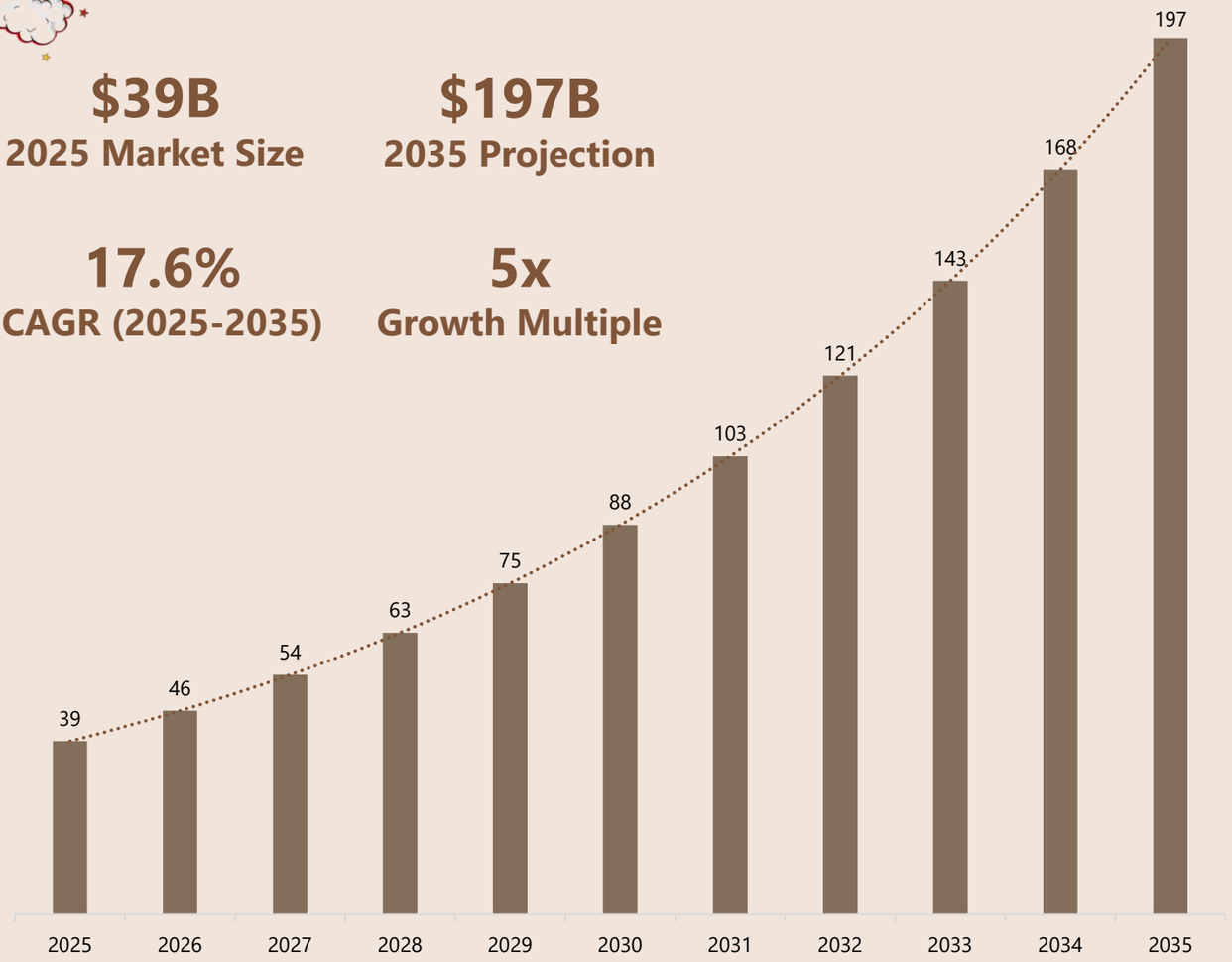
Explosive Growth Trajectory

$39B
2025 Market Size

$197B
2035 Projection

17.6%
CAGR (2025-2035)

5x
Growth Multiple



Note: Values shown in the graph are estimated and represent approximate figures

Key Drivers

- **Patent Expiry of Blockbuster Biologics**
  $170B biologic revenues expected to lose exclusivity by 2030, creating a large opportunity for biosimilar launches
- **Healthcare Cost Containment Pressure**
  Biosimilars provide lower-cost alternatives to originator biologics, driving strong payer and government support
- **Rising Burden of Chronic Diseases**
  Increasing prevalence of oncology, autoimmune disorders and chronic conditions like diabetes is expanding demand for biosimilar therapies
- **Expanding Regulatory Support**
  Regulatory bodies such as the EMA, FDA and Health Canada have established stringent guidelines for biosimilar evaluation and approval, supporting faster market adoption
- **Growing Industry Investment & Pipeline Expansion**
  Major pharma and biotech players like Sandoz are increasing investments and partnerships to develop biosimilar portfolios

Key Takeaways

- **Strong Double-Digit Market Expansion**
  The biosimilars market is projected to grow at ~17-18% CAGR, reaching ~$190B-$200B by the mid-2030s, driven by cost-effectiveness of biosimilar drugs
- **Patent Cliff Creating Major Market Opportunity**
  An estimated $170B in biologic drugs will lose exclusivity by 2030, opening a substantial pipeline opportunity for biosimilar entrants
- **Shift Toward High-Value Therapeutic Areas**
  Oncology and autoimmune diseases dominate biosimilar adoption, with monoclonal antibodies emerging as the largest and fastest-growing segment

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Mapping Biosimilars Market Trends (1/4)

Regulatory Trends

- **2025** was a breakthrough year of Denosumab biosimilars, with multiple approvals referencing PROLIA & XGEVA
- 20 Denosumab biosimilars were approved in 2025 across the US (8) & EU (12), significantly increasing global market competition & biosimilar adoption
- On **Oct'25**, FDA approved OMYLCO (omalizumab-igec), as the first & only interchangeable biosimilar to Xolair
- As of **Jul'25**, US FDA approved Kristy (Insulin Aspart-xjhz), as the first & only interchangeable rapid-acting biosimilar with NovoLog (Insulin Aspart)
- The US FDA is streamlining biosimilar development, were robust analytical, functional & PK data may reduce the need for comparative clinical efficacy (CES) or large-scale switching studies
- Indian government is strengthening biosimilar regulations to align with global standards and ensure high-quality research & manufacturing as these therapies become key treatments for cancers & autoimmune diseases
- WHO encouraging biosimilar adoption to improve access to biologics

Therapeutics Portfolio Trends

- The surge of biosimilars into oncology & autoimmune segments is expected to accelerate the future growth of US biosimilars market
- **Oncology** biosimilars of HERCEPTIN, AVASTIN & RITUXAN are gaining market momentum due to high treatment expenses & reimbursement driven demand
- **Immunology** biosimilars (for biologics such as HUMIRA, ENBREL, and REMICADE) are witnessing rapid adoption as an increasing number of alternatives enter the market
- Expansion into diabetes (INSULIN biosimilars), **ophthalmology** (LUCENTIS, EYLEA biosimilars) & **neurology** (AIMOVIG, BOTOX biosimilars) is anticipated to significantly expand the market
- **Endocrinology** Biosimilars of Denosumab (reference brands: PROLIA & XGEVA) are gaining momentum in the US and EU due to rising osteoporosis and oncology-related bone disorder burden, alongside strong cost-reduction and reimbursement-driven adoption

Price Trends

- **Oncology**
  ASPs **declined across biosimilars**, driven by price reductions and competitive discounts among key products
- **Supportive Care**
  Mixed pricing trend-filgrastim biosimilars declined, while pegfilgrastim biosimilars increased
- **Immunology**
  Mostly stable pricing, with a slight increase in infliximab biosimilars and a minor decline in tocilizumab biosimilars
- **Ophthalmology**
  Highly stable pricing, with only a marginal decline in ranibizumab biosimilars

Source: Samsung Bioepis Q1'26 Report

ASP: Average Selling Price

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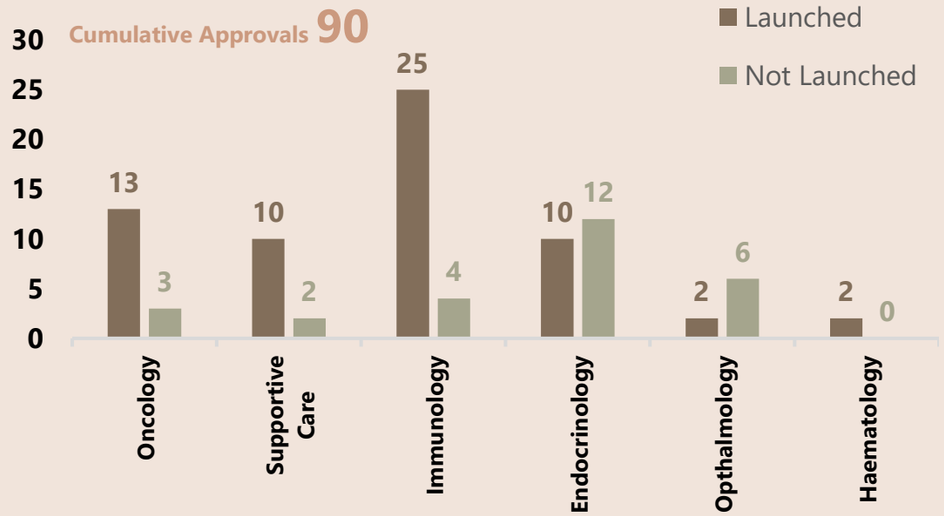
Mapping Biosimilars Market Trends (2/4)

US

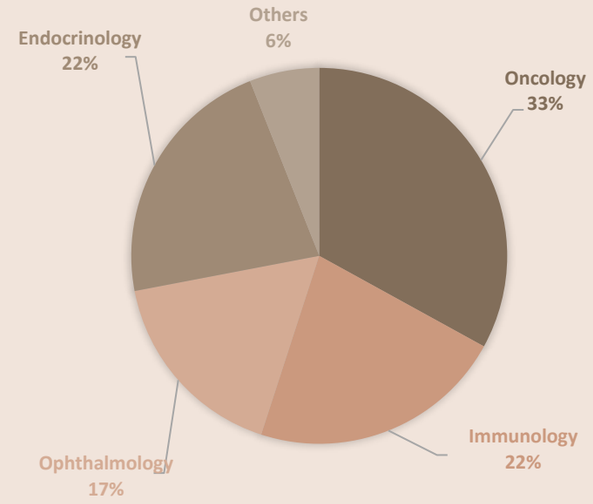


- **Approvals:** As of Dec 2025, US FDA has approved 90 biosimilars across 20 distinct biologic molecules, of which 63 (70%) have launched in the US market. The therapeutic landscape for biosimilars is rapidly evolving, with immunology and oncology together representing majority share of FDA approvals
- **Regulatory:** Since 2015's first biosimilar approval, the US FDA has developed a streamlined framework prioritizing swift market access
  - Regulatory guidance updates continue to refine requirements (e.g., potential reductions in human clinical studies required for approval of certain biosimilars and cut development costs)
- **Saving Impact:** Biosimilars have saved the US healthcare system $56 billion to date. On average, ASP decreased by 52% within five years of the first biosimilar launch, with more mature markets achieving even greater price reductions over time (up to 77%).

Biosimilars Approval & Launch Status in US (2006-2025)



Biosimilar Approvals by Therapeutic Area in US (2025)



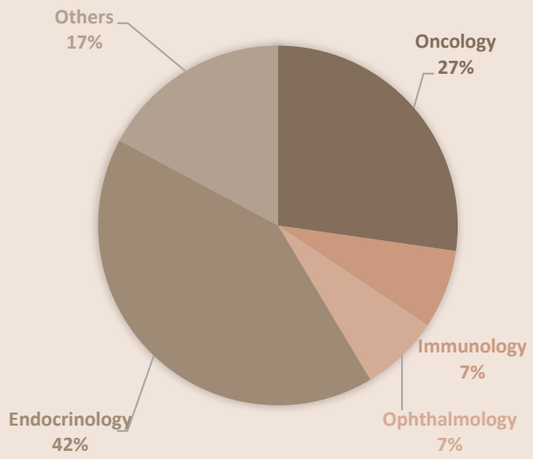
ASP: Average Selling Price

Mapping Biosimilars Market Trends (3/4)

EU

- **Saving Impact**
  As of **Jul 2024**, cumulative list price savings reached €56B from the impact of biosimilar competition in Europe
- **Adoption**
  Biosimilars directly influence the accessible biologics market, account for 12% of biologic treatment volume, driving cost savings through lower prices than reference biologics
- **Regulatory Landscape**
  In **2025**, CHMP approved 41 biosimilars, marking the highest number of approvals to date
- **Centralized Approval**
  EMA reviews and approves biosimilars under centralized procedures (unless otherwise specified),ensuring consistent standards with reference medicinal product

Biosimilar Approvals by Therapeutic Area in EU (2025)



India

- By **Dec 2025**, India has approved 146 biosimilars (recombinant therapeutics products), making it one of the countries with largest **biosimilar portfolios** worldwide
- India is transitioning from "Pharmacy of the World" to an Innovation Incubator, with companies like Biocon Biologics securing major international distribution deals in 2025
- India has emerged as a major biosimilar hub, supported by regulatory initiatives such as “Make in India” program, with the biosimilars market expected to grow significantly
- India government announced “Biopharma SHAKTI” initiative aims to strengthen the country biosimilars ecosystem by investing ₹10,000 Cr over 5 years
- **Major players:** Biocon, Intas Pharmaceuticals, Dr. Reddy’s Laboratories, Zydus Lifesciences, Reliance Life Sciences, Hetero Biopharma & Lupin hold several biosimilar approvals in different therapeutic areas

APAC

- **Asia Pacific (APAC) growth**
  The growing pharmaceutical sector in APAC is boosting investments in biotechnology and biosimilar research. Countries such as China, India, and South Korea are emerging as major global hubs due to their cost-efficient research environments and skilled workforce
- **Regulatory harmonization**
  Governments across Asia Pacific are strengthening regulatory frameworks for biosimilars to streamline approvals and ensure safety & efficacy. Harmonization of guidelines with global standards set by EMA & FDA further enhancing market confidence
- **Key Countries:**
  - Significant investment in biotech firms with improved regulatory pathways through China NMPA, drive the growth of the biosimilars market
  - South Korea is a leader in biosimilar innovation, with companies such as Celltrion and Samsung Bioepis driving the growth of the country biosimilars market
- **Healthcare expansion**
  Rising healthcare expenditures & insurance coverage in APAC are increasing demand for affordable biologic treatments, with biosimilars offering cost-effective alternatives for a growing patient population

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Market Trends in Emerging Therapy Area (4/4)

Biosimilars are no longer confined to oncology and immunology; emerging therapy areas are reshaping the competitive landscape from 2026–2030

1 OPHTHALMOLOGY-ANTI-VEGF PIPELINE

- Ophthalmology (notably anti-VEGF therapies) is viewed by payers as a high-burden area where increasing biosimilar competition is expected to drive greater savings as more products launch over the coming years (2025-2030)
- The pipeline includes several anti-VEGF biologics including high-dose Aflibercept (EYLEA HD), ophthalmic Bevacizumab-vikg (LYTENAVA), the antibody-biopolymer conjugate Tarcocimab tedromer (KSI-301)

2 NEUROLOGY / MULTIPLE SCLEROSIS

- Polpharma Biologics' TYRUKO (Natalizumab biosimilar) the first and only EMA- and FDA-approved MS biosimilar was developed entirely in-house and launched in the US in November 2025, demonstrating the feasibility of complex CNS biosimilar development and setting the stage for pipeline expansion into other MS biologics such as Ocrelizumab (PB018 in Polpharma pipeline)

3 RARE DISEASE-COMPLEMENT-INHIBITOR

- New biosimilar programs are moving into very high-cost complement inhibitors, such as Samsung Bioepis' EPYSQLI (Eculizumab), now approved in the US, EU and other markets as a SOLIRIS biosimilar for PNH and aHUS, with additional development toward gMG, positioning complement-inhibitor biosimilars to broaden access to rare-disease biologics and open a new rare-disease opportunity segment

4 GASTROENTEROLOGY/IMMUNOLOGY-VEDOLIZUMAB & NEXT-WAVE BIOSIMILARS

- Alvotech is conducting a P3 trial of AVT16 (Vedolizumab biosimilar) in moderate-to-severe ulcerative colitis, while Polpharma Biologics is advancing PB016, both targeting the ENTYVIO franchise with potential approvals expected around 2027+
- In January 2026, at the J.P. Morgan Healthcare Conference, Samsung Bioepis announced six new biosimilar candidates (Dupilumab, Guselkumab, Ixekizumab, Vedolizumab, Ocrelizumab, and fam-trastuzumab deruxtecan-nxki) added to its pipeline expanding its early-stage programs across immunology, dermatology and oncology indications

5 ENDOCRINOLOGY- DIABETES/OBESITY

- Endocrinology particularly GLP-1 agonists and other metabolic agents, is flagged as an emerging "next wave" for biosimilars as these biologics become major cost drivers; TRULICITY (Dulaglutide) and related molecules are cited as strategic future biosimilar targets in the 2028–2030 period and beyond
- Market analyses project continued growth in insulin biosimilar use through 2030, driven by pricing pressure and access needs, while Boan Biotech's BA5101, a Dulaglutide biosimilar to TRULICITY has completed P3 in China and is under BLA review representing the first filed dulaglutide biosimilar globally

PNH: Paroxysmal nocturnal hemoglobinuria, aHUS: atypical Hemolytic uremic syndrome, gMG: generalized Myasthenia gravis

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Market Entry Barriers

Swathi



Biosimilar adoption is constrained by a combination of regulatory, legal, commercial, manufacturing, and behavioral barriers that collectively delay market entry and limit competitive uptake



Barrier	Impact	Severity	Region
Regulatory Complexity: Extensive comparability studies, clinical pharmacology programs, and immunogenicity data	Development cost & timeline	High	Global
Patent Thickets: Reference product manufacturers file numerous secondary patents on formulations, devices, manufacturing processes, and dosing regimens beyond the core molecule patent. Eg. 250+ patents for Adalimumab, 100+ for etanercept	Market entry timing	High	Global
Rebate Walls: Originator manufactures offer conditional rebates of 50–70% to PBMs contingent on exclusive or preferred formulary status. Net price dynamics make competition on list price alone insufficient for access	Formulary access	High	US
Physician Inertia: Physicians often resist switching stable patients to biosimilars due to nocebo effects, concerns about immunogenicity, and lack of familiarity	Market penetration	Medium	US
Data Exclusivity: Regulatory data exclusivity (12 years in US, 8+2+1 in EU) prevents biosimilar manufacturers from referencing originator dossiers until expiry	Earliest entry timing	Medium	US/EU
Co-pay Accumulator Programs: PBMs increasingly use accumulator adjustment programs that prevent originator co-pay card values from counting toward patient deductibles	Patient switching	Medium	US
Patient Support Infrastructure: Building infrastructure for nurse education, injection training, adherence programs, financial assistance is expensive and lags originator capabilities	Patient experience	Low	US
Group Purchasing Organizations (GPO) Contracting hold long-term supply agreements with reference manufacturers	Hospital channel access	Medium	US

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Nancy

Biosimilars Market Key Takeaways

- *Biosimilar approvals surged in 2025, led by multiple denosumab biosimilars in the US & EU*
- *Regulatory agencies are simplifying biosimilar pathways, reducing reliance on large clinical studies lowering development costs*
- *Biosimilar portfolios are expanding beyond oncology & immunology into diabetes, ophthalmology & neurology*
- *Increasing competition is influencing biosimilar pricing, with notable price declines in oncology biosimilars*
- *US market continues to expand, with 90 FDA-approved biosimilars across 20 biologic molecules*
- *Europe remains a mature biosimilar market, delivering significant healthcare cost savings*
- *India is emerging as a biosimilars hub through policy initiatives & portfolio expansion*
- *Asia Pacific is becoming a global biosimilar hub, driven by regulatory harmonization, investment& cost advantages*
- *Rising healthcare spending in APAC is boosting demand for affordable biosimilar therapies*

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Where Deals Meet Opportunity?

- **Biosimilar Transactions**
- **Upcoming Exclusivity Losses**



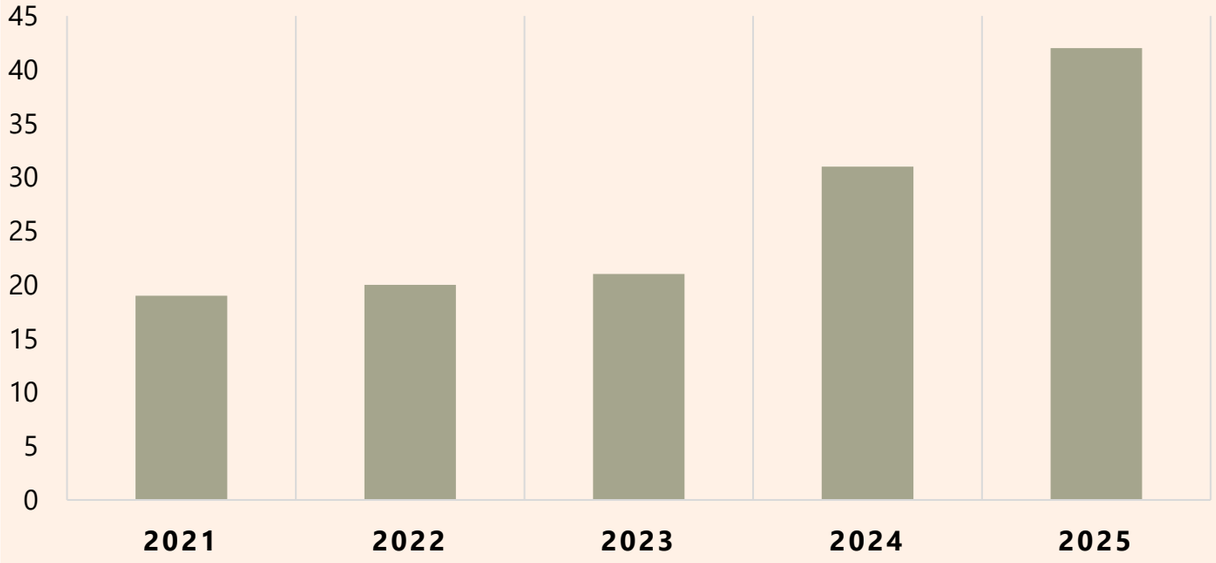
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Ruchi

Biosimilar Deal Activity Surges

Timeline Visual: Jan'21-Feb'26

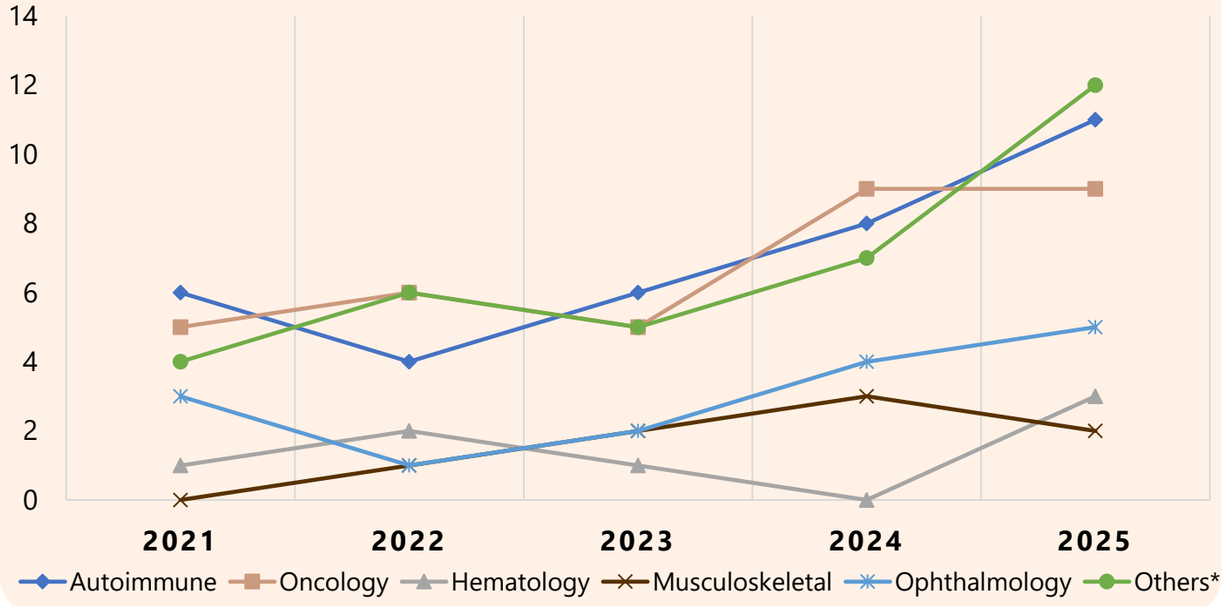
Year-on-Year Growth Trajectory



From 2021-2023, biosimilar deals increase only gradually, reflecting early but cautious market adoption, before a clear inflection in 2024 when dealmaking becomes more strategic and mainstream, culminating in 2025 with the strongest growth in the series and a rapidly expanding, high-confidence biosimilar partnering environment.

In 2026, 2 deals were signed for oncology and ophthalmology in January and February respectively, with undisclosed values.

Biosimilar Investments Across Therapy Areas



Across 2021 to 2025, Oncology investment rises steadily from about 5 to 9 deals, showing that investors see it as a reliable area for long-term value. In contrast, Autoimmune investment really takes off after 2023, jumping from roughly 6 to 11 deals by 2025, which points to a rapidly growing space were moving early can still secure strong margins before competition intensifies.

*Others: Dermatology, Endocrine/ Metabolic, Gastrointestinal, Infectious, Inflammation, Pulmonary, Nephrology

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WHY BIOSIMILAR DEALS ???

Grow Year-on-Year

01 A wave of patent expiries is approaching, especially for blockbuster biologics in oncology, immunology, and other chronic disease areas. As these therapies lose exclusivity, the door opens wider for biosimilar players, creating fresh opportunities for new market entrants, strategic partnerships, and competitive growth

02 With healthcare costs rising, payers and health systems are prioritizing lower-cost biologics. Biosimilars are becoming central to access decisions, tender strategies, and cost-containment efforts across global healthcare markets

03 Many companies lack comprehensive global commercialization capabilities, prompting them to pursue licensing, co-development and regional distribution partnerships to expand market reach, optimize resources, and effectively share financial and operational risks

04 Companies in India, Korea, and China are strengthening biosimilar capabilities, driving outbound partnerships for regulated markets and in-licensing for domestic portfolios, significantly contributing to rising global deal activity and cross-border collaborations

05 Regulatory agencies such as the US FDA and EMA have streamlined biosimilar approval pathways, enhanced review experience, and clarified interchangeability and substitution guidelines, thereby reduced development uncertainty and lowered overall approval risk for manufacturers

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Upcoming Exclusivity Losses (1/2)

Ruchi

USA
Europe
Initial Approval
Loss of Exclusivity
Intravenous
Subcutaneous

Company Name	Product Name	2009-2013	2014-2018	2019-2023	2024-2028	2029-2033	2034-2038	2039-2043
Lilly	trulicity (once weekly)		2014 – 2027					
			2014 – 2029					
	CYRAMZA (ramucirumab injection)		2014 – 2026					
			2014 – 2028					
	taltz (ixekizumab) injection		2016 – 2030					
			2016 – 2031					
	Emgality (Galcanezumab)		2018 – 2033					
			2018 – 2033					
	omvoh (mirikizumab-mrkz)			2023 – 2037				
				2023 – 2038				
	Ebglyss (lebrikizumab-lbkz)			2024 – 2036				
				2024 – 2033				
	kisunla (donanemab-azbt)			2024 – 2036				
				2025 – 2036				
MERCK	KEYTRUDA (pembrolizumab) injection 100 mg		2014 – 2028					
			2015 – 2031					
	WINREVAIR (sotatercept-csrk) for injection 45 mg, 60 mg			2024 – 2027				
				2024 – 2026				
	ENFLONSIA (clesrovimab-cfor) 105 mg injection			2025 – 2039				

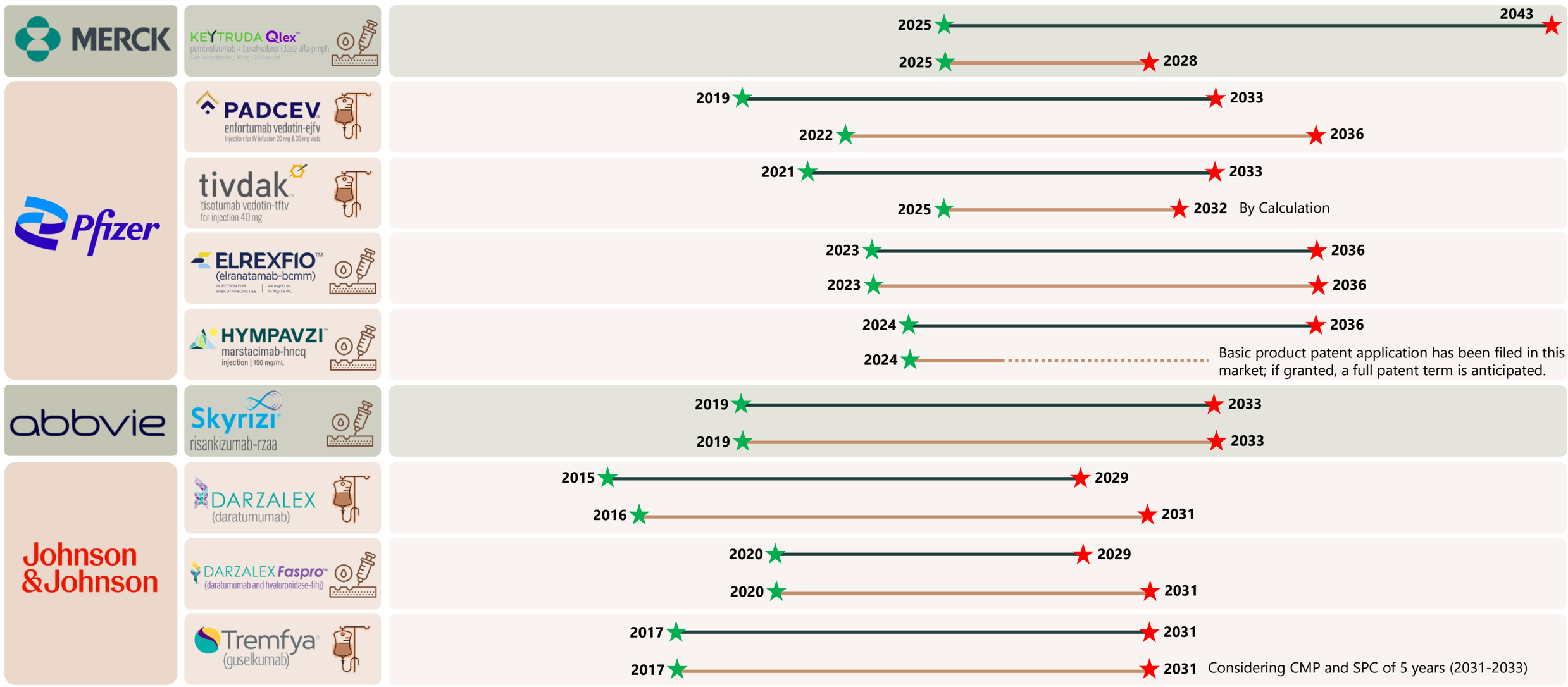
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Upcoming Exclusivity Losses (2/2)

Ruchi

USA
Europe
Initial Approval
Loss of Exclusivity
Intravenous
Subcutaneous



Conclusion:

The cliff risk for these five players is highly staggered: legacy IV assets like TRULICITY and CYRAMZA hit LOE first (mid-2020s), while newer SC/IV launches (EMGALITY, SKYRIZI, ELREXFIO, TREMFYA, KEYTRUDA SC/Qlex, DARZALEX FASPRO, OMVOH, KISUNLA) are largely insulated until 2031–2043, creating a long "patent ladder" rather than a single drop.

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Who's In the Game?

- Mapping the Approved Biosimilar Landscape
- Market Leader



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Approved & Launched Biosimilars
In Across Therapeutic Areas (US) (1/4)

Bably

THERAPY AREA	ONCOLOGY				HEMATOLOGY	MUSCULOSKELETAL
MOLECULE	Bevacizumab	Trastuzumab	Rituximab	Pertuzumab	Eculizumab	Teriparatide
PARENT PRODUCT	AVASTIN bevacizumab — Genentech — 2004	Herceptin trastuzumab — Genentech — 1998	Rituxan Rituximab — Genentech, Biogen — 1997	PERJETA pertuzumab — Genentech — 2012	SOLIRIS (eculizumab) — ALEXION — 2007	FORTEO teriparatide injection — Lilly — 1987
	MVASI (bevacizumab-awwb) — AMGEN BIOSIMILARS — 2017	Ogivri (trastuzumab-dkst) — Biocon — 2017	Truxima (rituximab-abbs) — teva, CELLTRION — 2018	POHERDY® — ORGANON — 2025	Epysqli (eculizumab-aagh) — SAMSUNG BIOEPIS — 2024	BONSITY teriparatide injection — Alvogen — 2025
	Zirabev bevacizumab-bvzr — Pfizer — 2019	Herzuma (trastuzumab-pkrb) — teva — 2018	Ruxience rituximab — Pfizer — 2019		BKEMV (eculizumab-aeeb) — AMGEN BIOSIMILARS — 2024	
	Alymsys (bevacizumab-maly) — amneal — 2022	Ontruzant trastuzumab-dttb — MERCK — 2019	RIABNI (rituximab-arrx) — AMGEN BIOSIMILARS — 2020			
	Vegzelma bevacizumab-adcd — CELLTRION — 2022	Trazimera trastuzumab-qyyp — Pfizer — 2019				
	AVZIVI — 百奥泰 BIO-THERA — 2023	KANJINTI (trastuzumab-anns) — AMGEN BIOSIMILARS — 2019				
	Jobevne (bevacizumab-nwg) Injection — Biocon — 2025	HERCESSI (trastuzumab-strf) — accord BioPharma — 2024				

Launched

Approved

Intravenous

Subcutaneous

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Approved & Launched Biosimilars
In Across Therapeutic Areas (US) (2/4)

Bably

THERAPY AREA | **ENDOCRINOLOGY**

MOLECULE | **Denosumab**

PARENT PRODUCT

prolia (denosumab injection) — AMGEN — 2010	XGEVA (denosumab) injection — AMGEN — 2010
jubbonti (denosumab-bbdz) injection 60 mg/mL — SANDOZ — 2024	Boncresa (denosumab-mobz) — amneal — 2025
WYOST (denosumab-bbdz) injection 120 mg/1.7 mL — SANDOZ — 2024	Oziltus (denosumab-mobz) — amneal — 2025
OSPOMYV™ — SAMSUNG BIOEPIS — 2025	BILDYOS (denosumab-nxxp) injection 60 mg/mL — Henlius ORGANON — 2025
Xbryk® — SAMSUNG BIOEPIS — 2025	BILPREVDA (denosumab-nxxp) injection 120 mg/1.7 mL — Henlius ORGANON — 2025
Stoboclo (denosumab-bmwo) — CELLTRION — 2025	Enoby™ (denosumab-qbde) — hikma. — 2025
Osenvelt — CELLTRION — 2025	XTRENBO™ (denosumab-qbde) — hikma. — 2025
Conexxence denosumab-bnht — FRESENIUS KABI — 2025	Bomyntra denosumab-bnht — FRESENIUS KABI — 2025
Bosaya™ (denosumab-kyqq) — Biocon — 2025	AUKELSO — Biocon — 2025
OSVYRTI® (denosumab-desu) — Biocon — 2025	JUBEREQ® (denosumab-desu) — Biocon — 2025

Launched

Approved

Intravenous

Subcutaneous

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Approved & Launched Biosimilars
In Across Therapeutic Areas (EU) (3/4)

Bably

THERAPY AREA	ONCOLOGY				HEMATOLOGY
MOLECULE	Bevacizumab		Trastuzumab		Rituximab	Eculizumab
PARENT PRODUCT	AVASTIN bevacizumab — Genentech — 2005		Herceptin trastuzumab — Genentech — 2000		MabThera Rituximab — Genentech — 1998	SOLIRIS (eculizumab) — ALEXION — 2007
	MVASI (bevacizumab-awwb) — AMGEN BIOSIMILARS — 2018	AVZIVI — FGK REPRESENTATIVE SERVICE — 2024	Herzuma (trastuzumab-pkrb) — CELLTRION — 2017	Tuznue — teva PRESTIGE BIOPHARMA — 2024	Truxima (rituximab-abbs) — CELLTRION — 2017	Epysqli (eculizumab-aagh) — SAMSUNG BIOEPIS — 2023
	Zirabev bevacizumab-bvzr — Pfizer — 2019	Vegzelma bevacizumab-adcd — CELLTRION — 2022	Ontruzant trastuzumab-dttb — SAMSUNG BIOEPIS — 2017	Dazublys (trastuzumab) — CuraTeQ — 2025	RIXIMYO — SANDOZ — 2017	BEKEMV — AMGEN BIOSIMILARS — 2023
	Alymsys (bevacizumab-maly) — mAbxience — 2021		Trazimera trastuzumab-qyyp — Pfizer — 2018	HERWENDA — SANDOZ — 2023	Rixathon — SANDOZ — 2017	
	Aybintio bevacizumab — SAMSUNG BIOEPIS — 2020		Ogivri (trastuzumab-dkst) — Biocon — 2018		Blitzima Rituximab — CELLTRION — 2017	
	OYAVAS bevacizumab — STADA — 2021		KANJINTI (trastuzumab-anns) — AMGEN BIOSIMILARS — 2018		Ruxience rituximab — Pfizer — 2020	
	Abevmy — BCIL — 2021		Zercepac — accord Make it better — 2020		ITUXREDI — Dr.Reddy's — 2024	

Launched

Approved

Intravenous

Subcutaneous

BCIL Biosimilar Collaborations Ireland Ltd

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Approved & Launched Biosimilars
In Across Therapeutic Areas (EU) (4/4)

Bably

THERAPY AREA	MUSCULOSKELETAL	ENDOCRINOLOGY
MOLECULE	Teriparatide	Denosumab
PARENT PRODUCT	FORSTEO Teriparatid Injection — Lilly — 2003	prolia (denosumab) injection — AMGEN — 2010
		XGEVA (denosumab) injection — AMGEN — 2010
	Terrosa teriparatide — Gedeon Richter Ltd. — 2017	jubbonti (denosumab-bbdz) injection 60 mg/mL — SANDOZ — 2024
		JUNOD — Gedeon Richter — 2025
		WYOST (denosumab-bbdz) injection 120 mg/1.7 mL — SANDOZ — 2024
		YAXWER — Gedeon Richter — 2025
	Movymia Teriparatide — STADA — 2017	OBODENCE — Samsung Bioepis — 2025
		BILDYOS (denosumab-nxxp) injection 60 mg/mL — Science Pharma part of JD group — 2025
		Xbryk® — Samsung Bioepis — 2025
		BILPREVDA (denosumab-nxxp) injection 120 mg/1.7 mL — Science Pharma part of JD group — 2025
	LIVOGIVA — Theramex — 2020	Stoboclo (denosumab-bmwo) — CELLTRION — 2025
		ROLCYA — SANDOZ — 2025
		Osenvelt — CELLTRION — 2025
		VEVZUO — BCIL — 2025
	teriparatide Sondelbay strength for independence — accord Make it better — 2022	Conexxence denosumab-bnht — Fresenius Kabi — 2025
		ZADENVI — ZENTIVA — 2025
		Bomyntra denosumab-bnht — Fresenius Kabi — 2025
		ENWYLM — ZENTIVA — 2025
		IZAMBY — mAbxience — 2025
		DENBRAYCE — mAbxience — 2025
		OSVYRTI (denosumab-desu) — accord Make it better — 2025
		JUBEREQ (denosumab-desu) — accord Make it better — 2025

- Launched
- Approved
- Intravenous
- Subcutaneous
- BCIL — Biosimilar Collaborations Ireland Ltd

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Shubham

Company Profile: Sandoz (1/2)

Leading the Biosimilar Revolution & Generic Innovation

Company Snapshot

$11.1 B
Revenue FY'25

23,000+
Committed Employees

100+
Countries Served

1,300+
Product Portfolio

13
Marketed Biosimilars

30%
Biosimilar Sales Contribution

Octavus Insights

- Sandoz is emerging as a key leader in the upcoming $660B biosimilar opportunity wave (2026–2035), supported by a strong development portfolio covering ~59–65% of value potential (27 biosimilars)
- Biosimilars are now the company's primary growth driver, contributing ~30% of revenue with double-digit growth and structurally higher margins than generics, supporting its targeted EBITDA margin expansion from 21.7% to ~24–26% by 2028

Geographic Presence

Headquarters
Basel, Switzerland

Global Reach
Operations in more than 100 countries worldwide

Financial Highlights

- Company reported solid financial growth in 2025, with net sales increasing to $11.1B from $10.4B in 2024 (5% constant-currency growth)
- Biosimilars contributed 30% of total sales in 2025
- In 2025, Sandoz issued new bonds and secured a $2B credit facility to strengthen its financial position

Sales (2025)



Golden Decade of Opportunity (2026-2035)

~$660B Originator drug sales losing exclusivity

$200B Sales opportunity from Sandoz's current pipeline (27 molecules)

>$300B Branded biologics losing patent protection

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Shubham

Company Profile: Sandoz (2/2)

Leading the Biosimilar Revolution & Generic Innovation

Robust Development Pipeline

>400
Generic assets in development, addressing ~$220B in originator sales

27
Biosimilars currently in development, covering ~59% of biosimilar LoE value opportunities

65%
Coverage of generic loss-of-exclusivity (LoE) opportunities by value

5
Assets in regulatory review or approaching launch

Recent & Upcoming Launches (2025-2026)

Major Biosimilar Launches 2025

- US
  - PYZCHIVA (ustekinumab)
  - TYRUKO (natalizumab)
  - WYOST & JUBBONTI (denosumab)
- EU
  - AFQLIR (aflibercept)
  - PYZCHIVA autoinjector

Key Generic Launches 2025

- Iron sucrose
- Rivaroxaban
- Enoxaparin sodium

Expected Launches 2026

- US
  - CIMZIA (certolizumab pegol) re-launch (Early 2026)
  - AFQLIR (aflibercept) (Q4 2026)

Pipeline Highlights

- **Trastuzumab (HERWENDA):** Under regulatory review in the US
- **Bevacizumab (AVZIVI):** Under regulatory review in the US and EU
- **Insulin Analogs:** Including glargine, aspart, lispro under regulatory review
- **Pembrolizumab (KEYTRUDA biosimilar):** In clinical development
- **Nivolumab (OPDIVO biosimilar):** In clinical development phase
- **Development Portfolio:** 7 assets in technical development, 9 in early development, and 6 in clinical development

GLP-1 Strategy

Sandoz plans early GLP-1 launches in select markets from 2026, expansion into the US and Europe with targeted products by 2031–2035, followed by next-generation incretin therapies after 2035 to sustain growth and competitiveness

Strategic Partnerships 2025

- PYZCHIVA (ustekinumab) launched Q1 2025, primarily via private-label strategy in partnership with Samsung Bioepis
- In 2025 signed an agreement with Henlius to commercialize an ipilimumab biosimilar
- In April 2025, Sandoz decided to minimize the P3 study for its pembrolizumab (KEYTRUDA biosimilar) in line with regulatory discussions and evolving biosimilar development frameworks which significantly reduce development timelines and costs

Source: Sandoz Annual Report 2025

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Evolving Regulatory Frameworks



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Prashant

Regulatory Shifts:

How Biosimilars are Transforming Specialty Pharmacies

Background:
The Biologics Price Competition and Innovation Act (BPCIA) created a pathway in the US for biosimilars, allowing lower-cost alternatives to expensive biologic drugs in specialty pharmacies.
Initially, biosimilars gained traction as more approvals were granted. Later, stronger FDA guidelines improved interchangeability, making it easier to use them for disease treatments.
Over time, biosimilars were used more widely for immune-related conditions, while FDA actions helped lower development costs, boosting competition and expanding their use in oncology and autoimmune therapies.

2010

The policy foundation

In the US, the BPCIA opened the door for biosimilars in the pharmaceutical market

2015

The Race in the Market Begins

The first biosimilar was approved and launched, starting a new phase in drug pricing

2016-2018

Expanding Access, Driving Adoption

Biosimilars started to include monoclonal antibody, and changes in payment and insurance policies helped more patients get access to these treatments

2019-2021

Stronger Confidence Through Clear Regulations

The FDA updated its guidelines and set final rules for when biosimilars can be safely substituted for original medicines i.e. **interchangeability**

2022-2024

The Next Wave of Biosimilars for Common Diseases

Biosimilars became available to treat inflammatory diseases

2025

FDA Aims to Speed Biosimilars and Cut Drug Costs

FDA announced draft guidance to streamline biosimilar development by **reducing unnecessary clinical efficacy studies and switching studies**, **relying more on analytical testing.** This aims to cut costs/time, boost competition against high-cost biologics and increase market share

2010 2015 2016 2017 2018 2019 2020 2021 2022 2023 2024 2025

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Streamlining Regulations, Accelerating Biosimilars

Case Studies:

- On April 30, 2025, Sandoz announced plans to minimize its P3 trial for a Keytruda biosimilar after discussions with US FDA and **new EMA guidance (Streamlining development and assessment of biosimilar medicines)**
- On Feb 17, 2025, Formycon announced that it will terminate the P3 "Lotus" trial for its Keytruda biosimilar FYB206 after US FDA discussions, relying instead on Phase 1 data and analytical evidence for approval

FDA Moves to Accelerate Biosimilar Development and Lower Drug Costs
Eliminating unnecessary clinical trials

Global Harmonization
Non-US reference products are now accepted by using comparative clinical data

Cutting unnecessary regulations and providing clear guidance to speed up approvals and reduce uncertainty

As of 2024, 9 interchangeable biosimilar were approved without additional switching study data

Facilitating pharmacy-level substitution
Advancing **interchangeability** to empower pharmacists to substitute lower-cost biosimilars, removing barriers similar to generic drug substitution





Streamlining development and assessment of biosimilar medicines
Eliminating unnecessary clinical trials

Global Harmonization
Simplifying regulations for sponsors using non-EU/EEA data at certain study stages

All approved biosimilars are considered interchangeable in EU, though individual member states can set their own criteria, with EMA encouraging a coordinated approach

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Biosimilar Market Access & Adoption Barriers



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Pricing & Commercial Strategies

Swathi

Successful biosimilar commercialization depends on competitive pricing, strong payer access, and building physician confidence through robust clinical evidence and value-added support programs



Aggressive Discount

Pricing biosimilars 30-60% below the originator WAC helps secure exclusive or preferred formulary placement, sacrificing margins to rapidly gain market volume, particularly in tender-driven markets

Value/Outcomes Based

Rebates tied to real-world switching outcomes and treatment persistence help differentiate biosimilars but require strong contracting systems and real-world data infrastructure. Growing in US Medicaid and EU horizon-scanning frameworks

Partnerships

Strategic partnerships with distributors, hospitals, and health systems to expand reach

Tender-Specific Bidding

Customize pricing, bundling, and service packages per tender lot in government or hospital procurement markets

Brand Differentiation

Product differentiation through improved drug delivery devices (auto-injector), patient support services, enhanced packaging or storage, and digital adherence tools, as seen with biosimilars of Insulin Glargine

Physician engagement

Companies drive biosimilar adoption by building physician confidence through clinical evidence, real-world data, and targeted medical education

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Swathi

Market Pricing and Reimbursement Landscape

Biosimilars operate in a uniquely complex pricing environment shaped by the interplay of reference product discounts, payer incentives, and market competition.

EU has faster biosimilar uptake than the US due to single-payer national health system, tenders, substitution policies, and lack of rebate-wall dynamics

Key Market Dynamics	US	EU
Substitution	FDA interchangeability designation	Rare
Pricing model	ASP-based; WAC rebates; PBM contracts	Tender/ Rebated list
Key payers	CMS, PBMs, IDNs, GPOs	National health systems
HTA requirement	Not required; PBM evidence packages	Rare - budget impact focus
Uptake drivers	Formulary exclusivity	Tender policy

ASP: Average Sales Price; WAC: Wholesale Acquisition Cost; PBM: Pharmacy Benefit Managers; CMS: Centers for Medicare & Medicaid Services; IDNs: Integrated Delivery Networks; GPOs: Group Purchasing Organizations

US

Market access is dominated by complex network of PBMs, private insurers and Medicare system

Medicare Part B (Physician-Administered)
Reimbursement for originators : ASP +6% of originator ASP
Reimbursement for qualifying biosimilars (IRA provision through 2027): ASP +8% of the reference product's ASP

Medicare Part D (Pharmacy-Dispensed)
Rebate system historically favored branded biologics because PBMs could extract larger rebates from originators fighting for formulary placement.
Biosimilar manufacturers have less rebate room.
Recent reforms (IRA, rebate rule discussions) are slowly reshaping this

Commercial Payers
Adoption varies widely. Some payers mandate biosimilar use (step therapy or non-medical switching policies); others allow incumbency to persist due to existing rebate contracts with originator manufacturers

EU

EU is widely regarded as the global leader in biosimilar adoption

- **Single-payer systems, mandatory price-linkage, tender competition, and no interchangeability switching-study requirement** create a fundamentally more biosimilar-friendly environment
- Reimbursement is a member state competency each with distinct pricing rules, procurement models, and physician incentive structures
  - ✓ In France, biosimilar prices are set at 40% below the originator
  - ✓ Italy mandates a 20% discount on originator prices
  - ✓ Germany employs a regulated pricing policy, but differs in its reliance on retail-level tendering rather than a centralized tendering framework
  - ✓ UK operates under an unregulated pricing policy with NHS setting caps for high-cost biosimilars while tenders drive competition via flexible discounts

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Swathi

US Case Study: Humira Biosimilar Adoption Challenges

The 2023 adalimumab biosimilar launches exemplified PBM rebate traps blocking uptake, but 2024-2025 shifts toward biosimilar favouritism accelerated market erosion for Humira

Initial Launch Barriers:

- Nine adalimumab biosimilars entered the US in January 2023 at list price (WAC) discounts of 55-85% versus Humira, **yet captured only <2% market share in one year**
- PBMs (Express Scripts, CVS Caremark, OptumRx) retained Humira as exclusive preferred on national commercial formularies due to **high rebates**

PBM Strategic Pivot

- External pressures from employers, policymakers, and the public over the lack of transparency and misaligned incentives in the rebate system prompted 2024 formulary overhauls
- CVS Caremark excluded Humira from its 2024 national commercial formularies, favoring its Cordavis private-label biosimilar (Sandoz-manufactured, ~85% off Humira WAC). This was followed by Express Scripts and OptumRx causing Humira's dominant market share to plummet almost overnight

For 2025, Humira almost vanished from PBMs' standard formularies.

Humira and Its Biosimilars, Price and PBM Formulary Coverage, 2025

Product name	Manufacturer	WAC (2024)	WAC vs. Humira	Express Scripts	CVS Caremark	Optum Rx
Humira	AbbVie	$6,922	n.a.	–[4]	–[5]	–/✓[6]
Humira	Cordavis[7] (AbbVie)	$6,922	–0%	–	–	–
Simlandi[1,2]	Alvotech/Teva	$1,038	–85%	✓	–	–
adalimumab-ryvk[1,2]	Quallent[8] (Alvotech/Teva)	$3,750	–46%	✓	–	–
Amjevita[1] (High WAC)	Amgen	$6,576	–5%	–	–	✓[3]
Amjevita[1] (High WAC)	Nuvaila[9] (Amgen)	$6,576	–5%	–	–	–
Amjevita[1] (Low WAC)	Amgen	$3,115	–55%	–	–	–
Amjevita[2] (Low WAC)	Amgen	$1,385	–80%	–	–	–
Amjevita[2] (Low WAC)	Nuvaila[9] (Amgen)	$1,385	–80%	–	–	✓
Hulio	Biocon Biologics	$6,576	–5%	–	–	–
adalimumab-fkjp	Biocon Biologics	$995	–86%	–	–	–
Cyltezo[1,2]	Boehringer Ingelheim	$6,576	–5%	✓	–	–
adalimumab-adbm[1,2]	Boehringer Ingelheim	$1,315	–81%	–	–	–
adalimumab-adbm[1,2]	Quallent[8] (Boehringer Ingelheim)	$3,750	–46%	✓	–	–
Yuflyma[2]	Celltrion	$6,576	–5%	–	–	–
Yusimry[2]	Meitheal[10]	$995	–86%	–	–	–
Idacio	Fresenius Kabi	$6,576	–5%	–	–	–
adalimumab-aacf	Fresenius Kabi	$899	–87%	–	–	–
Abrilada[1] (High WAC)	Pfizer	$6,576	–5%	–	–	–
Abrilada[1,11] (Low WAC)	Pfizer	$1,038	–85%	–	–	–
Hadlima[2]	Samsung Bioepis/Organon	$1,038	–85%	–	–	–
Hyrimoz[1,2]	Sandoz	$6,576	–5%	–	✓	–
Hyrimoz[1,2]	Cordavis[7] (Sandoz)	$1,300	–81%	–	✓	–
adalimumab-adaz[1,2]	Sandoz	$1,315	–81%	✓	✓	–

WAC = wholesale acquisition cost

1. Indicates product is interchangeable with Humira reference product for one or more product strength.
2. Indicates product is available in high concentration formulation.
3. Amjevita (high WAC) is on Optum Rx's Premium and Select formularies, but excluded from its Premium Value formulary. Boehringer Ingelheim's adalimumab-adbm is on the Premium Value formulary.
4. As of January 2025, Express Scripts excluded Humira for new patients. As of July 2025, Express Scripts will exclude Humira from its preferred national formulary for all utilizers.
5. In April 2024, CVS Caremark removed Humira from its major commercial formularies. The reference product remains on CVS Caremark's Choice and Standard Opt Out commercial formularies, which account for a small share of its customers and covered lives.
6. As of January 2025, Optum Rx excluded Humira from its Premium Standard formulary, but includes it on tier 3 with prior authorization on its Select Standard formulary.
7. Cordavis is a subsidiary of CVS Health, which also operates CVS Caremark. The co-branded product made by AbbVie is not a biosimilar, because it is supplied by the manufacturer of the reference product.
8. Quallent Pharmaceuticals is a subsidiary of Cigna's Evernorth business, which also operates Express Scripts.
9. Nuvaila is a subsidiary of Optum, which also operates Optum Rx. The Nuvaila products launched in 2025.
10. In 2024, Coherus divested Yusimry to Hong Kong King-Friend Industrial Co., which markets the product via its Meitheal subsidiary.

Source: Drugchannels.net

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Production and Developmental Hurdles

Swathi

Manufacturing

Cell Line & Expression System
Biosimilar production must replicate innovator molecules, with challenges in glycosylation, clonal stability, and analytical comparability

Process Sensitivity
Changes to biomanufacturing steps such as a different vector, cell screening system, culture media, production or purification method, or excipients can impact a product's safety and efficacy

Scale-Up Complexity
What works at lab scale often doesn't translate linearly to commercial-scale bioreactors, requiring re-optimization

Purification & Formulation
Designing chromatographic purification is challenging due to lack of standardization

Raw Material Variability
Complex biological raw materials vary lot-to-lot, adding another layer of variability to control



Developmental

Extensive Characterization Burden
Regulators require detailed analytical comparison of biosimilars and reference products across key structural and functional attributes, including sequence, structure, activity, purity, and impurities

Functional Assays
Links structural similarity to clinical performance; developing sensitive, reproducible bioassays for complex biologics is technically challenging

Clinical & Regulatory Pathway
Regulators may require PK/PD and efficacy studies depending on molecule complexity, increasing development cost and time. Lack of a harmonized global regulatory framework further complicates multi-market approvals

Immunogenicity Risk
Minor structural or formulation differences can trigger unwanted immune responses that affects safety and efficacy

Intellectual Property Landscape
Originator companies filing secondary patents covering formulations, dosing regimens, and manufacturing processes to delay competition years beyond the expiration of the primary patent

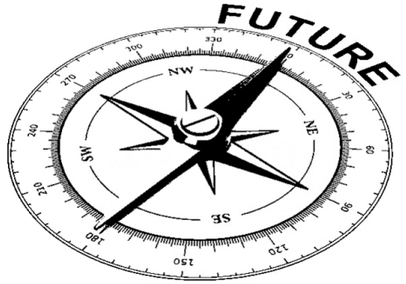
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Future Direction



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Future Potential of Biosimilar

Biosimilar 2.0

Background:
Biosimilar 2.0 is transforming the market through advanced technologies like next-gen sequencing and AI-driven bioprocessing, enabling high-quality biosimilars for complex therapies. Global regulatory harmonization and patient-focused evidence boost confidence and safety. With strong growth in oncology and immunology, led by markets like China, combined with cost reductions from pricing competition and CDMO efficiencies, Biosimilar 2.0 offers significant market expansion and attractive investment opportunities

Key Advancements

- High analytical similarity via next-gen sequencing, AI-driven bioprocessing, and high-throughput screening for complex mAbs
- Holistic evaluation with rigorous non-clinical, PK/PD, and clinical studies for safety/efficacy, plus global regulatory harmonization
- Patient-centric designs incorporating real-world evidence and interchangeability studies to boost confidence

Market Growth

- The biosimilars market is projected to grow at ~17-18% CAGR, reaching ~$190B-$200B by the mid-2030s, driven by cost-effectiveness of biosimilar drugs
- Oncology biosimilars driving growth: $9.5B in 2026 to $37B by 2034 (18.5% CAGR)
- China leads adoption with policies like Volume-Based Procurement (VBP), saving billions in cancer care
- Europe & North America follow with accelerated approvals and strategic partnerships

Strategic Drivers

- Patent cliffs for blockbuster biologics accelerate 2025-2026 approvals in immunology and oncology
- CDMOs drive biosimilar cost savings via scaled bioprocessing, enabling 50-60% price drops compared to generics. Newer tech boosts 50% reductions in emerging markets

Implications

- From 2025-2034, 118 biologics face patent expiry, unlocking a $232-234B market for biosimilars
- Attractive for investors with focus on high-margin, complex pipelines and China/global expansion
- Biosimilar 2.0 represents a scalable, technology-powered growth frontier that delivers affordability, broader patient access, and strong commercial upside for strategic investors

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Hashtags to Headlines

Transforming Social Noise into Clear Insights

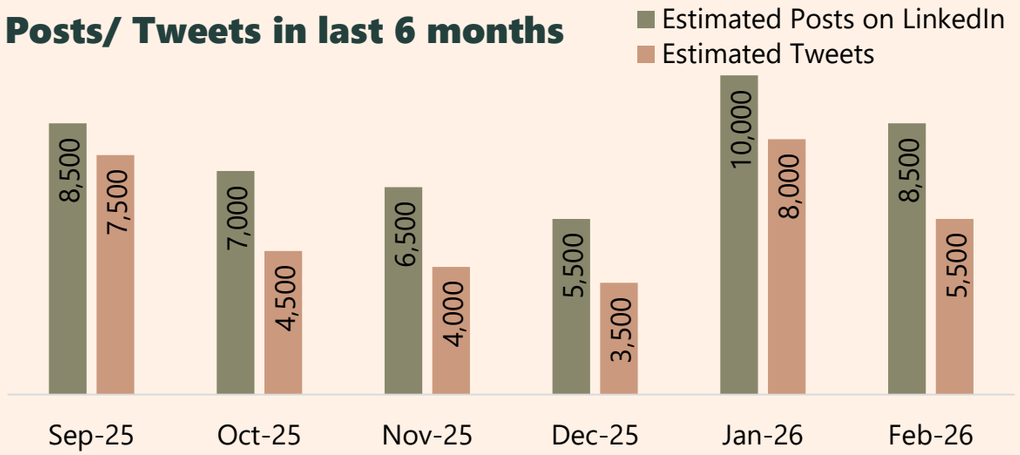


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This presentation is non-exhaustive. For additional details or clarifications, please feel free to contact us at bd@octavusconsulting.com

Sentimental Analysis

Posts/ Tweets in last 6 months



These post and tweet counts are based on the presence of the keywords: "biosimilar," "biosimilars," #biosimilar, or #biosimilars.

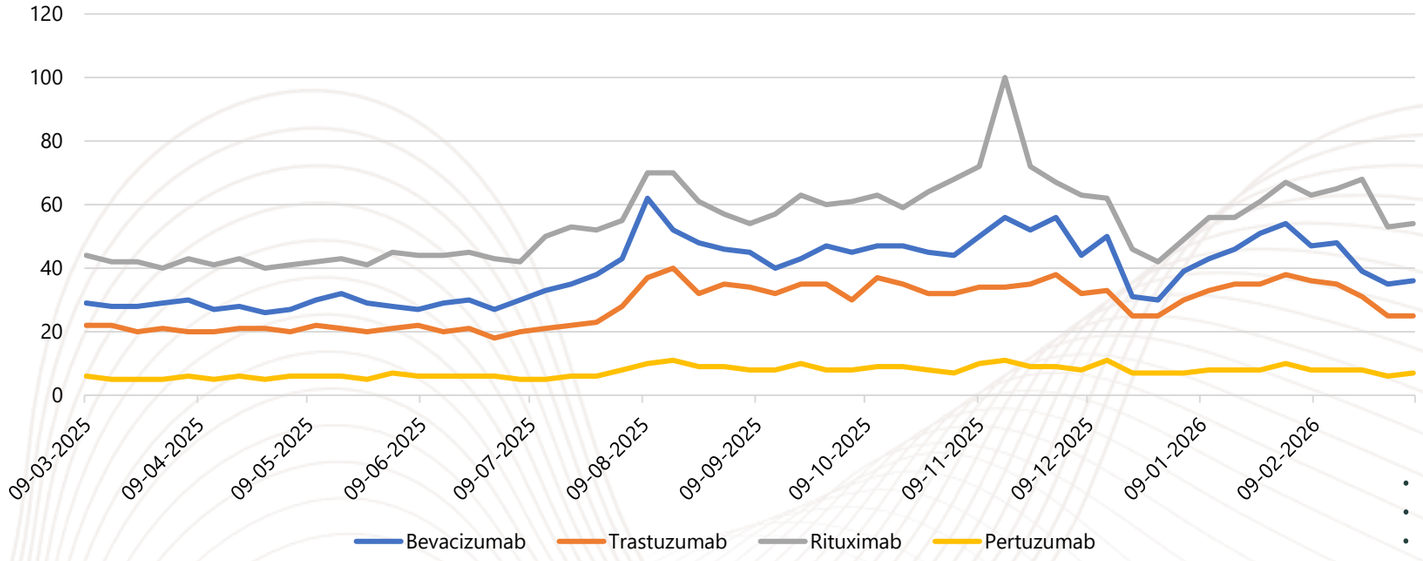
LinkedIn:

- Spike in posts on denosumab (PROLIA/XGEVA) biosimilar approvals and EMA reflection paper
- Heavy Q3 earnings digestion for Sandoz, Samsung Bioepis and other biosimilar leaders
- "Banner year" 2025 recap with 20+ FDA biosimilar approvals and 2026 outlook
- Peak chatter around JPM with CEOs selling the "Golden Decade" and interchangeability wins

Twitter:

- Peak buzz: Denosumab biosimilar approvals plus EMA clinical trial reflection paper
- Strong focus on MENA build-out, especially Hikma-Celltrion moves
- Continuous IRA / Medicare Negotiation threads tied to biosimilar pricing strategy
- Year-end threads on record 18+ FDA biosimilar approvals in 2025
- #JPM26 live-tweets on the "Golden Decade" narrative and 2026 pipeline trackers

Google Trends Based on Worldwide Web Search



The oncology biosimilar landscape shifted from **market establishment** to **aggressive disruption** during this six-month window. While established molecules like **Trastuzumab, Bevacizumab, and Rituximab** reached a mature "utility" phase - maintaining high search volume driven by 50–70% price discounts - **Pertuzumab** emerged as the primary trend-breaker.

The **Nov'25 FDA approval of POHERDY** (the first interchangeable Pertuzumab biosimilar) triggered a massive search spike, signaling a new era of "auto-substitution" in breast cancer care. This was amplified by an **October 2025 FDA draft guidance** that streamlined the path to interchangeability by reducing the need for costly switching studies.

By **Jan'26**, the focus at JPM shifted toward the "Next Wave," with companies like **Biocon** and **Samsung Bioepis** unveiling pipelines for PD-1 inhibitors (Nivolumab and Pembrolizumab). Overall, the period was defined by a transition from simply asking "What is a biosimilar?" to operationalizing **interchangeability** and high-value oncology pipelines.

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KOL Analysis

1. Key Voice & Leadership Positioning: The posts from Richard Saynor, CEO of Sandoz, his messaging consistently highlights the future of affordable medicines and expanding patient access, aligning Sandoz with healthcare sustainability and innovation

2. Core Messaging Themes: The social media content emphasizes three major themes:

- **Access to Affordable Medicines**

  Posts stress that biosimilars can significantly expand patient access to life-changing treatments, particularly for diseases such as cancer, rheumatoid arthritis, and Crohn's disease

- **Market Opportunity for Biosimilars**

  Saynor highlights that biosimilars currently represent less than ~7% of the global biologics market, signaling a major growth opportunity over the next decade

- **Policy & Regulatory Advocacy**

  The content supports regulatory reforms, such as reducing unnecessary clinical trials and improving patent frameworks to accelerate biosimilar adoption

3. Strategic Narrative: The narrative presented across posts is that the next decade will be pivotal for biosimilars. With many biologics losing patent protection, biosimilars can drive:

- Healthcare cost savings
- Expanded treatment access
- Reinvestment into innovation

This positions Sandoz as ready to lead the next wave of affordable biologics globally.

4. Engagement & Audience Interaction: Audience engagement appears supportive and aligned with the message. Comments from industry professionals reinforce the view that:

- Biosimilars are safe and effective alternatives
- The current market penetration is still too low
- Greater adoption is necessary to maximize healthcare system benefits

5. Brand & Reputation Impact: The social media activity strengthens Sandoz's reputation in two ways:

- Industry leadership in biosimilars
- Advocacy for healthcare affordability



Richard Saynor

CEO of Sandoz, a global leader in generic and biosimilar medicines. He has led the company since 2019 and brings over 20 years of pharmaceutical industry experience across generics, biosimilars, and established medicines.

Sandoz
1,121,205 followers
3mo •
+ Follow

The future of affordable medicines is unfolding right now.

🗓️ Today at the FT Live Global Pharma and Biotech Summit in London, our CEO Richard Saynor discussed the decade ahead: over USD 600 billion in annual medicine sales will lose exclusivity between now and 2035. That's more than any comparable period in history, split roughly equally between generics and biosimilars.

For originator companies, this represents disruption. For patients and healthcare systems worldwide, it's the largest opportunity ever to expand access to life-changing treatments.

Here's what that looks like in practice:

🔷 Generics already prove the model works. Worldwide, they deliver 80% of medicines used at less than a third of the total cost. That's not slowing down.
🔷 Biosimilar medicines are just getting started. Treatments for cancer, autoimmune diseases, and chronic conditions that once cost healthcare systems tens of thousands per patient will become accessible to millions more. The FDA's recent decision to streamline approval pathways accelerates that timeline.

Challenges persist. Patent abuses, supply chain vulnerabilities for critical medicines like antibiotics, and reimbursement barriers still stand in the way. But momentum is building.

The next decade will redefine what's possible when healthcare systems can invest savings into innovation, infrastructure, and expanding care. This is the future we're pioneering.

#PioneeringAccess #FTPharma

Richard Saynor • 2nd
CEO Sandoz
3mo • Edited •
+ Follow

At yesterday's FT Live Global Pharma and Biotech Summit in London, I was asked about the biggest opportunity in pharma over the next decade.

My answer?

Getting biosimilar medicines into the hands of patients who need them most. Right now, biosimilar medicines represent less than 7% of the global biologics market, despite proven safety and efficacy.

That has to change.

The FDA's recent decision to eliminate unnecessary clinical trials for biosimilar approvals is a step forward, but we need bolder action on the patent environment and price sustainability to deliver the true potential of the many biologics losing patent protection over the next 10 years.

The stakes are high. We're talking about treatments for cancer, rheumatoid arthritis, Crohn's disease – conditions that affect millions globally.

When healthcare systems save billions on treatment, that money can fund innovation, expand access and treat more patients. It's not one or the other.

The next 10 years will determine whether we deliver on the promise of affordable biologics at scale. At Sandoz, we're ready to lead that charge.

#FTPharma Patrick Temple-West

Fethi Trabelsi • 3rd+
Senior Life Sciences Executive | Operations, Strategy & Transformation
3mo

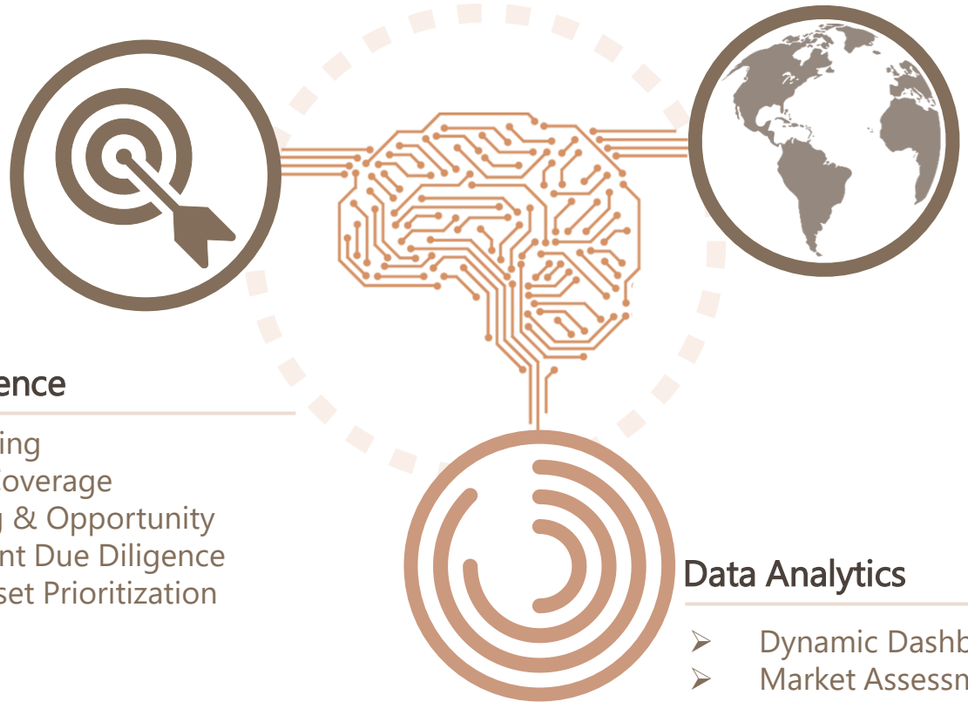
Richard Saynor 100% in agreement that 5% of the global market is not enough. Biosimilar medicines proved to be as safe and as efficient than the brands.

Like | Reply

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ABOUT COMPANY

Octavus delivers evidence-based consulting services to empower our clients in making well-informed decisions.



Market Research

- Quantitative & Qualitative Research
- Awareness, Trial & Usage (ATU) Studies
- KOL & PAG Identification & Mapping
- Concept/Product Testing
- AI & Human Confluence Transcription
- Content Analysis & Translation

Business Intelligence

- CI & Monitoring
- Conference Coverage
- Market Sizing & Opportunity
- Pre-investment Due Diligence
- Company/Asset Prioritization

Data Analytics

- Dynamic Dashboarding & Reporting
- Market Assessment & Epidemiology Analysis
- Sales Reporting & In-line Forecasting
- Social Media Analytics
- Clinical Analytics



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Month: March 2026

Presenter: Himanshu Sehgal

